Fulvestrant · injections every 4 weeks

NCCP schedule: 500 mg on Days 1 and 15 of the first 28-day cycle, then Day 1 of each 28-day cycle. This means Day 29 next, then every 28 days.

Hormone therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Injection area

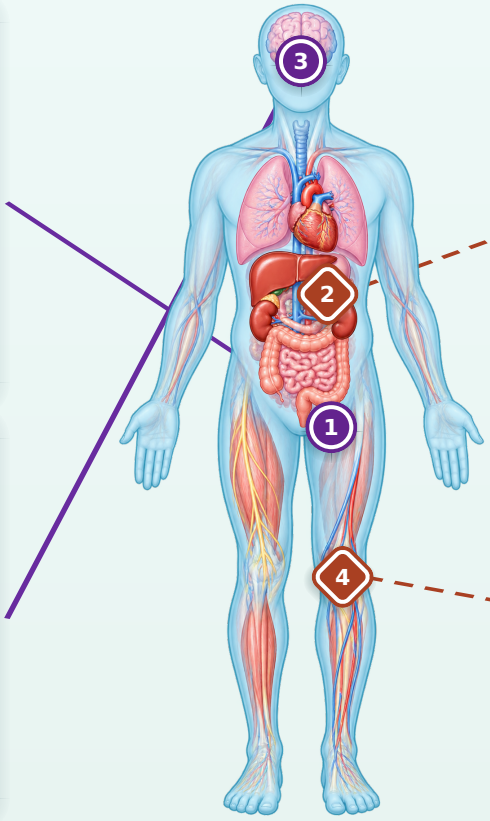
Fulvestrant

Report severe gluteal pain, numbness or pain travelling down the leg.

2 Liver

Fulvestrant

Report jaundice, dark urine or marked new illness; keep indicated liver tests.



3 Hot flushes

Fulvestrant

Tell the team if flushes or sweats disturb sleep and daily life.

4 Joints / muscles

Fulvestrant

Report persistent pain or symptoms limiting activity.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Tell the team about unusual bleeding or extensive bruising, injection-site pain with numbness or pain down the leg, jaundice or worsening symptoms. Severe chest pain, breathlessness or stroke symptoms require emergency help. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

YOUR PRACTICAL TREATMENT PLAN

Know the next step.

Taking treatment

NCCP schedule: 500 mg on Days 1 and 15 of the first 28-day cycle, then Day 1 of each 28-day cycle. This means Day 29 next, then every 28 days.

Each usual 500 mg dose is given as two 5 mL injections, one in each buttock, slowly over one to two minutes per injection. This is not an intravenous infusion; no port or pump is needed for fulvestrant itself.

Checks and follow-up

Before treatment, check blood count, kidney and liver function, the menopausal and treatment history, pregnancy exclusion and bleeding risk. Before injections, tell staff about anticoagulants, bruising, bleeding or previous nerve symptoms. Repeat liver tests when indicated; scans follow the agreed cancer-control plan. Tell anyone ordering an oestrogen blood test that you receive fulvestrant because some tests can give a misleadingly high result.

The whole-treatment map distinguishes early-breast follow-up from advanced-disease response assessment.

Support between reviews

Make injection visits manageable

Discuss injection discomfort, flushes, joint or muscle symptoms, fatigue and nausea. Ask early for symptom help or practical support attending appointments.

Support medicines

Fulvestrant alone does not require routine steroid premedication, injections to raise white cells or a standard anti-sickness course. A combination medicine can add its own supportive-care needs.

Bleeding and nerve symptoms

Tell staff about blood thinners, unusual bruising or bleeding before an injection. Severe buttock pain, new numbness, weakness or pain down the leg needs prompt assessment.

Tests and appointments

Confirm the extra loading visit and later appointments, who books required blood tests and scans, and who reviews the results. Tell the laboratory team about fulvestrant if an oestrogen blood test is ordered.

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Tell the team about unusual bleeding or extensive bruising, injection-site pain with numbness or pain down the leg, jaundice or worsening symptoms. Severe chest pain, breathlessness or stroke symptoms require emergency help.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

Your complete treatment reference

https://sactcheck.com/patient/00361/

Public information only. No personal information is encoded.